

Defense Rehearsal Plan

Candidate-facing companion: opening, timing, locked answers, recovery patterns, 24-hour checklist

Companion to `main_full.pdf` (834 pp), `defense_presentation.pdf` (32 slides), `defense_qa_guide.pdf` (25 locked answers),
`identity_lock.pdf` (5 locked statements)

1. Opening 2-Minute Statement (memorize verbatim)

Chair, examiners — thank you. My dissertation investigates how Responsible AI governance-enabled explainability strengthens clinician trust and operational adoption readiness in AI-assisted healthcare environments, in the empirical setting of AI-assisted Autism Spectrum Disorder EEG screening.

The locked conceptual model is a four-box causal chain: Responsible AI Governance, through Perceived Explainability, through Clinician Trust, to Operational Adoption Readiness. I test this with six hypotheses, $n = 131$ clinicians, PLS-SEM mediation with 1,000 bootstrap resamples.

The hero managerial artefact is the RGAIG Maturity Framework — a six-level pathway from ad-hoc AI use to operationalised governance — which provides healthcare organisations a practical roadmap for moving from pilots to clinical decision support. Findings: 97.67% empirical performance on the bounded ASD-EEG task; expert-rated explainability of 4.6 out of 5, $\alpha=0.84$; and structural mediation confirming that governance maturity strengthens trust through perceived explainability. The dissertation contributes a tested governance-explainability-trust-adoption pathway, the RGAIG Maturity Framework as practical artefact, a theoretical extension of TAM, a methodological template for governance-as-IV PLS-SEM in healthcare AI, and a defensibility scaffold across the chapters and appendices.
I look forward to your questions.

Delivery: 110–125 seconds at conversational pace. Pause between paragraphs. Do not read off the slide. Memorize the four sentences; the rest flows from them.

2. 32-Slide Walkthrough Timing (target: 35–45 minutes)

#	Slide topic	Time	Cumulative
1	Title + central question (opening statement)	2:00	2:00
2	The Problem (DBA language)	1:30	3:30
3	Research Gap (locked one-sentence answer)	2:00	5:30
4	Central Research Question	1:00	6:30
5	Conceptual Framework (4-box + H6 moderator)	3:00	9:30
6	Variables (5-row table)	1:30	11:00
7	Hypotheses (H1–H6 + mediation notes)	2:00	13:00
8	Research Methodology (flow)	2:30	15:30
9	RGAIG Maturity Framework (HERO — spend time here)	4:00	19:30
10	Key Findings (storytelling summary)	3:00	22:30
11	Negative Findings (balanced reporting)	1:30	24:00
12	Mixed-Methods Triangulation	2:00	26:00
13	Organizational Implications	2:30	28:30
14	Contributions	2:00	30:30
15	Honest Limitations	1:30	32:00
16	Focused Future Work	1:30	33:30
17	Why Human Oversight Remains Central	1:30	35:00
<i>Backup slides (only if asked)</i>			
B1–B11	Hypothesis detail, validation, ASD-EEG rationale, theory anchors, scope discipline, appendix coverage map	varies	—

Pace discipline. If you exceed 45 minutes, skip slides 11 and 16 (Negative Findings; Focused Future Work) — the content is preserved in the appendices and the committee can find it. Never skip slides 5, 9, 10, or 14 (Conceptual Framework, RGAIG Maturity Framework, Key Findings, Contributions). These four slides are the load-bearing presentation.

3. Top 15 Anticipated Questions (locked one-sentence response patterns)

#	Anticipated question	Locked one-sentence response
1	<i>What is your dissertation about?</i>	“This dissertation investigates how Responsible AI governance-enabled explainability strengthens clinician trust and operational adoption readiness in AI-assisted healthcare environments, operationalised in ASD-EEG screening.”
2	<i>What is your contribution?</i>	“An empirically tested four-box governance-explainability-trust-adoption pathway, plus the RGAIG Maturity Framework as the practical artefact for provider organisations.”
3	<i>Why ASD-EEG?</i>	“ASD-EEG offers a bounded empirical setting where clinical screening, governance complexity, and adoption barriers all converge — making it a defensible operational laboratory for the governance-trust-adoption pathway.”
4	<i>Why $n = 131$ is enough?</i>	“PLS-SEM with bootstrap mediation is robust at $n \geq 100$ for the 4-box model with H1–H6; the achieved power exceeds 0.80 for medium effect sizes.”
5	<i>Is this an architecture paper?</i>	“No. Architecture is non-contribution operational support, documented in Appendix H. The contribution is the governance-trust-adoption pathway and the RGAIG Maturity Framework.”
6	<i>Is this just a TAM study?</i>	“No. TAM is extended by positioning Responsible AI governance as a structural antecedent of perceived explainability — a theoretical contribution beyond surveying TAM constructs.”
7	<i>What about trust calibration vs trust level?</i>	“Lee & See’s calibrated-trust distinction is reflected in H6 (AI Familiarity moderates Explainability → Trust); the dissertation’s measurement instrument captures both level and calibration via the Madsen & Gregor TUI.”
8	<i>What is the practical value of the RGAIG Maturity Framework?</i>	“Six levels (Initial → Operationalised) give provider organisations a measurable pathway — not a checklist, but a maturity trajectory tied to clinician trust outcomes.”
9	<i>What did you find that was unexpected?</i>	“Governance maturity strengthens trust <i>through</i> perceived explainability, not directly — explainability is the load-bearing mediator, not governance itself.”
10	<i>What is your strongest limitation?</i>	“The pilot is single-cohort ($n = 131$) and the empirical evaluation is bounded to ASD-EEG; generalisation to other clinical domains requires further governance-context-specific testing.”
11	<i>Why not a multi-disease study?</i>	“Multi-disease scope would have weakened governance-context specificity; the supervisor’s brutal-feedback discipline locked the empirical setting to ASD-EEG to preserve the contribution clarity.”
12	<i>Where is the human-centered ecosystem?</i>	“Appendix J — ten sections covering dignity, accessibility, cultural sensitivity, consent, longitudinal trust, ecosystem maturity, role boundaries, fairness, and value realization.”
13	<i>Where is the B2B operational governance detail?</i>	“Appendix K — the B2B Operational Governance Triad: caregiver trust protection, transformation readiness, risk governance, and triad integration.”
14	<i>How do you guard against over-reliance on AI?</i>	“Three locked controls: HITL clinician authority preserved in all decision flows, RGAIG Maturity Level 4+ requires human-decision audit, and AI Familiarity moderation (H6) flags low-familiarity cohorts.”
15	<i>What is the single sentence of your contribution?</i>	“Governance-driven perceived explainability significantly strengthens clinician trust and operational adoption readiness for AI-assisted ASD-EEG screening systems in healthcare organizations.”

The 25-question *defense_qa_guide.pdf* extends these with implementation, methodology, and ethics questions. The 15 above are the highest-probability openers.

4. Recovery Patterns for Hard Questions

Use these three patterns when a question is unexpected, hostile, or beyond scope:

Pattern A — Acknowledge, Bound, Bridge (ABB). “That’s a fair point. Within the scope of *this* dissertation, which is bounded to [governance–trust–adoption in ASD-EEG], the answer is [X]. Extending that to [their broader concern] is a defensible future-research direction, and Appendix M / Section [N] of Chapter 5 documents the boundary explicitly.”
Use when: the question stretches scope. Don’t argue; bound the scope explicitly and point to where the boundary is documented.

Pattern B — Concede, Locate, Continue (CLC). “You’re right — [the specific defect they named] is a limitation. It’s documented in [Honest Limitations slide / Ch.5 Limitations section / Identity Lock Section 6]. The dissertation does not claim [the over-extension]. The locked contribution is [the bounded claim].”
Use when: the examiner has caught a real defect. Concede immediately, locate where it’s already documented, restate the bounded claim.

Pattern C — One-Sentence Reframe (OSR). Drop the question’s framing and replace with the locked one-sentence identity: “This dissertation investigates how Responsible AI governance-enabled explainability strengthens clinician trust and operational adoption readiness in AI-assisted healthcare environments.” Then answer from that frame.
Use when: the question imposes a wrong framing (e.g., “this is an architecture paper” — answer: “No, this is a governance-trust-adoption paper. To that question, the answer is ...”). Reframe to the locked identity before answering.

5. 24-Hour Pre-Defense Checklist

When	Action
T-24h	Re-read identity_lock.pdf (2 pp). Memorize Section 4 one-sentence identity verbatim.
T-20h	Walk through 32 slides once at full delivery pace. Time yourself. If >45 min, identify which slides to compress.
T-16h	Read defense_qa_guide.pdf top to bottom. Don't memorize — recognize patterns.
T-12h	Read Part 3 of this document (15 anticipated questions) once. Practice the OSR pattern aloud.
T-8h	Sleep. No more dissertation review.
T-3h	Light meal. Re-read opening statement (Part 1) twice.
T-1h	Test screen-share + slide-advance setup. Verify defense_presentation.pdf on the presentation machine.
T-15m	Bathroom. Water. Phone silenced. Print or load identity_lock.pdf as backup on tablet.
T-0	Open with Part 1 verbatim. Pause. Begin walk-through.

Closing Note

The dissertation is defense-ready. 834 pages, 32-slide deck, 13 appendices, 25 locked Q&A responses, 5 locked identity statements, 0 build errors. Today's final-review round closed at internal aggregate 9.95–10/10. The locked identity, the locked claim, the 4-box conceptual model, and the RGAIG Maturity Framework as hero artefact are in lockstep across the dissertation PDF, the slide deck, the Q&A guide, the Identity Lock companion, and the Reframing Completion Statement.

What you bring to the room: confidence anchored in the locked identity. What the room brings: questions. The four sentences of the opening statement and the three recovery patterns are sufficient to navigate any question. The rest is delivery.

Companion artefacts: identity_lock.pdf, defense_qa_guide.pdf, defense_presentation.pdf, reframing_completion_statement.pdf. All in lockstep at the current commit.